

— THE REVIEWER'S EYE

An indemnity reviewer reads your notes in *five seconds.*

Here's what they look for. ↓

Screening, *recorded.*

BPE, soft tissues, the basics — and dated. “Did exam” is not a record.

Screening

RECORDED

Reasoning, *not findings.*

Show *why* this plan — the clinical thinking, not only what you saw.

Clinical reasoning

SHOWN

Options & *consent*.

Alternatives discussed — including no treatment — risks, and agreement.

Options & consent

DOCUMENTED

Advice & *response.*

“OHI given” is weak; record what you advised and how the patient engaged.

Advice & response

NOTED

Who, when,
contemporaneous.

Written at the time, attributable to a named clinician.

Who & when

SIGNED

Nosht

Five things. *Every note.*

Structured templates prompt all five —
defensible by default.

Nosht

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